

Policy on Credentialing and Privileging of Nursing Professionals

«Hospital Name»

Document control

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Prepared by	HR In-Charge / Personnel Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» runs a process for credentialing and privileging of nursing professionals permitted to provide patient care without supervision, so that only identified, qualified, and privileged nurses provide unsupervised care.

1. Purpose

This policy sets out how «Hospital Name» identifies nursing staff permitted to provide unsupervised patient care, documents and verifies their credentials, grants and communicates their privileges, and ensures they practise within those privileges.

This policy does not cover recruitment, induction, safety and quality training, or staff health and safety — those are covered in other hospital policies.

2. Scope

This policy applies to all nursing staff at «Hospital Name» permitted by law, regulation and the organisation to provide patient care without supervision.

3. Policy standards

«Hospital Name» identifies nursing staff permitted to provide patient care without supervision, in consonance with the Indian Nursing Council Act, 1947.

«Hospital Name» documents and periodically updates the education, registration, training and experience of nursing staff.

«Hospital Name» verifies information about nursing staff when possible.

«Hospital Name» grants nursing staff privileges in consonance with their qualification, training, experience and registration, reviewed at least annually.

«Hospital Name» ensures the requisite services a nursing professional is authorised to provide are known to that professional and to relevant departments.

«Hospital Name» ensures nursing professionals care for patients as per their privileging.

4. Non-negotiable rules

1. Do not let a nursing professional provide unsupervised patient care without first identifying them on the permitted list, in consonance with the Indian Nursing Council Act, 1947.
2. Do not leave a nursing professional's education, registration, training or experience undocumented or out of date.
3. Verify every nursing professional's credentials with the awarding organisation whenever possible.
4. Do not grant or allow privileges that are not in consonance with a nursing professional's qualification, training, experience and registration.
5. Do not leave a nursing professional or the relevant departments unaware of the services that professional is authorised to provide.
6. Do not let a nursing professional care for a patient outside their granted privileges.
7. Do not bypass the stop-work authority in Section 6 when the trigger conditions are met.
8. Staff who see a rule under this policy broken report it the same shift to the HR In-Charge / Personnel Officer or the Medical Superintendent.

5. What we do

5.1 Identification of permitted professionals

«Hospital Name» identifies nursing staff permitted by law, regulation and the organisation to provide patient care without supervision — individuals with the required qualification(s), training and experience, in consonance with the law (Indian Nursing Council Act, 1947). Providing unsupervised care outside this identified list is a stop-work trigger (Section 6).

5.2 Credential documentation

The education, registration, training and experience of nursing staff at «Hospital Name» are documented and updated periodically, after acquisition of new skills or qualification.

5.3 Credential verification

Information about nursing staff at «Hospital Name» is appropriately verified when possible, by checking with the organisation that awarded the qualification or training.

5.4 Granting of privileges

«Hospital Name» grants nursing staff privileges in consonance with their qualification, training, experience and registration — identifying what each nurse is authorised to do (for example, an infection-prevention-and-control nurse needs the requisite in-house or external training, experience, aptitude and knowledge for that role). Privileges are reviewed at least annually and revised where necessary. Granting or exercising privileges outside this record is a stop-work trigger (Section 6).

5.5 Communication of privileges

The requisite services a nursing professional at «Hospital Name» is authorised to provide are known to that professional, to nursing services and to the concerned departments, communicated internally.

5.6 Practising within privileges

Nursing professionals at «Hospital Name» care for patients as per their privileging. New staff may work under supervision until independent privileges are granted; the hospital maintains a mechanism confirming nursing professionals provide only the services they are privileged for.

6. Stop-work authority

Do not let a nursing professional provide patient care without supervision unless the HR In-Charge / Personnel Officer and the credentialing file show that professional identified and privileged for that care under this policy.

Stop-work applies to starting or continuing unsupervised care by that professional. Care already under way is handed to a privileged professional or brought under supervision; immediate life-saving care is not withdrawn while that handover happens.

The person who stops tells the HR In-Charge / Personnel Officer and the Medical Superintendent the same shift. Refusing to let an uncredentialed or unprivileged professional practise unsupervised is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
HR In-Charge / Personnel Officer	Owns this policy. Maintains the credentialing file and the identified-professional list.
Nursing Superintendent	Reviews nursing credentials and grants privileges; conducts the annual privilege review.
Medical Superintendent	Oversees implementation of this policy. Receives stop-work escalations.
All Staff	Report any nursing professional providing unsupervised care outside the identified or privileged list.

8. Quality monitoring

The HR In-Charge / Personnel Officer reviews credentialing files to confirm every nursing professional providing unsupervised care is identified and privileged. Documentary evidence is on file for each CORE objective element. Compliance with the stop-work authority in Section 6 is monitored through the credentialing file review. This policy itself is reviewed every year.

9. Training and staff acknowledgement

All staff covered by this policy are made aware of the credentialing and privileging process, including the stop-work authority in Section 6, and sign the acknowledgement below.

I have read the Policy on Credentialing and Privileging of Nursing Professionals of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

HR department; Nursing Superintendent's office; all department heads / unit in-charges; all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HR	Human Resources
HRM	Human Resource Management (NABH Hospitals 6th Edition chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition HRM.12

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required. Stop-work applies ONLY to the OEs marked 'Section 6 Stop-work' below.

OE	Level	Requirement	Where addressed	Responsible
HRM.12.a	CORE	Nursing staff permitted by law, regulation and the organisation to provide patient care without	Section 3; 5.1; Section 6 Stop-work	HR In-Charge / Personnel

		supervision are identified.		Officer
HRM.12.b	Commitment	The education, registration, training and experience of nursing staff are documented and updated periodically.	Section 3; 5.2	HR In-Charge / Personnel Officer
HRM.12.c	Commitment	The information about the nursing staff is appropriately verified when possible.	Section 3; 5.3	HR In-Charge / Personnel Officer
HRM.12.d	CORE	Nursing staff are granted privileges in consonance with their qualification, training, experience and registration.	Section 3; 5.4; Section 6 Stop-work	HR In-Charge / Personnel Officer
HRM.12.e	Commitment	The requisite services to be provided by the nursing staff are known to them as well as the concerned departments / units of the organisation.	Section 3; 5.5	HR In-Charge / Personnel Officer
HRM.12.f	Commitment	Nursing professionals care for patients as per their privileging.	Section 3; 5.6	HR In-Charge / Personnel Officer

13. Required Records / Evidence Checklist

HRM.12.a — Permitted nursing staff identified. [Stop-work trigger]

- Identified-nursing-professional list permitted to provide unsupervised patient care, with a qualification, training and experience verification record referencing the Indian Nursing Council Act, 1947.
- Cross-reference to the stop-work authority in Section 6 for unlisted practice.

HRM.12.b — Credentials documented and updated periodically.

- Education, registration, training and experience documentation record, updated periodically, with a personal-file record of each update.

HRM.12.c — Credentials verified when possible.

- Verification record with the awarding organisation, with a verification-completeness record.

HRM.12.d — Privileges granted in consonance with qualification, training, experience and registration. [Stop-work trigger]

- Granted-privilege record naming what each nurse is authorised to do, with an annual privilege-review record.
- Cross-reference to the stop-work authority in Section 6 for privileging outside this record.

HRM.12.e — Requisite services known to nurse and relevant departments.

- Communicated-service record to the nurse and to nursing services and concerned departments, with an internal-communication record.

HRM.12.f — Nursing professionals care as per privileging.

- Supervision record for new staff until independent privileges are granted.
- Mechanism-confirmation and privileging-compliance spot-check record.

14. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard HRM.12.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — HRM.12 interpretations.
- Indian Nursing Council. (1947). The Indian Nursing Council Act.
<https://www.indiannursingcouncil.org/uploads/files/16141575032256.pdf> — referenced in HRM.12's own interpretation as the statutory basis for identifying nursing staff permitted to provide care without supervision.
- Indian Nursing Council Act, 1947.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

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